

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

A data-driven overview for public health policy analysts and mental health program planners

Global data, 2023 • DALYs per 100,000 people

Source: Our World in Data — Mental Health; IHME Global Burden of Disease (2025)

Hundreds of Millions Affected

Lifetime depression risk is substantial, and mental health affects well-being, work, and relationships.

1 in 3 women

will experience major depression in their lifetime



1 in 5 men

will experience major depression in their lifetime

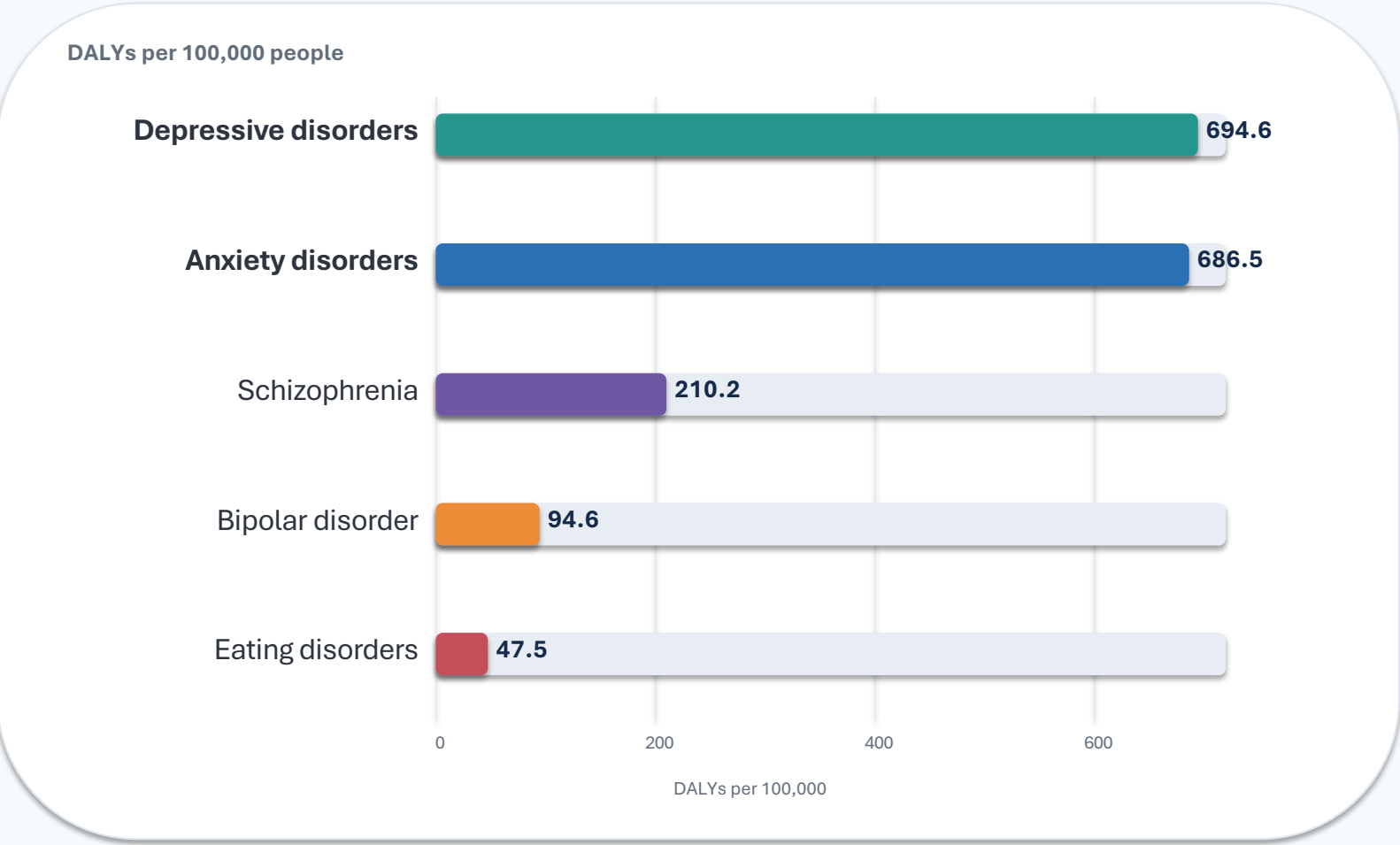


Why it matters

- Poor mental health affects well-being, ability to work, and relationships with friends, family, and community.
- Conditions span from common disorders such as depression and anxiety to less common but severe disorders such as schizophrenia and bipolar disorder.

Depression and Anxiety Dominate the Disease Burden

DALYs combine years of life lost and years lived with disability; rates shown per 100,000 people, World 2023.



Central finding

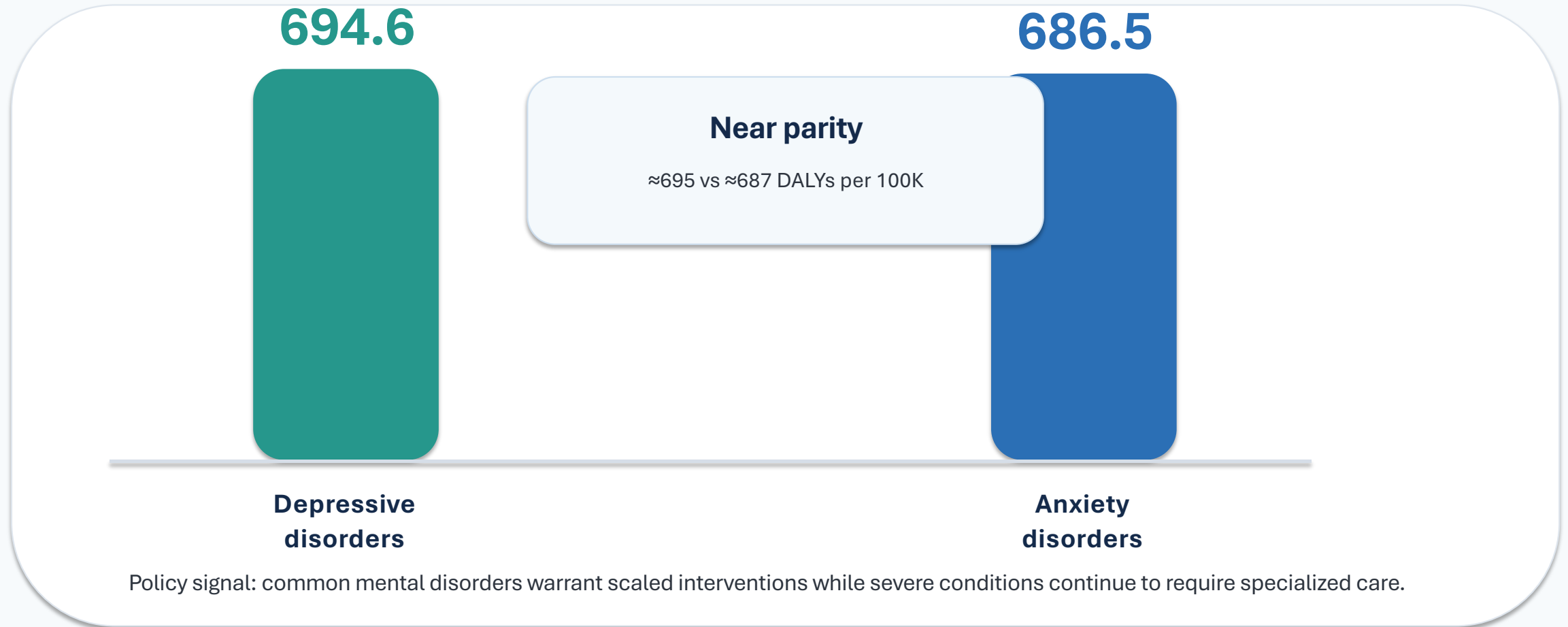
Depressive and anxiety disorders together dominate the global mental health burden, with nearly identical DALY rates.

Together, they account for roughly 80% of the total mental health disease burden across these five categories.

Figure reference: burden of disease from each category of mental illness, World, 2023.

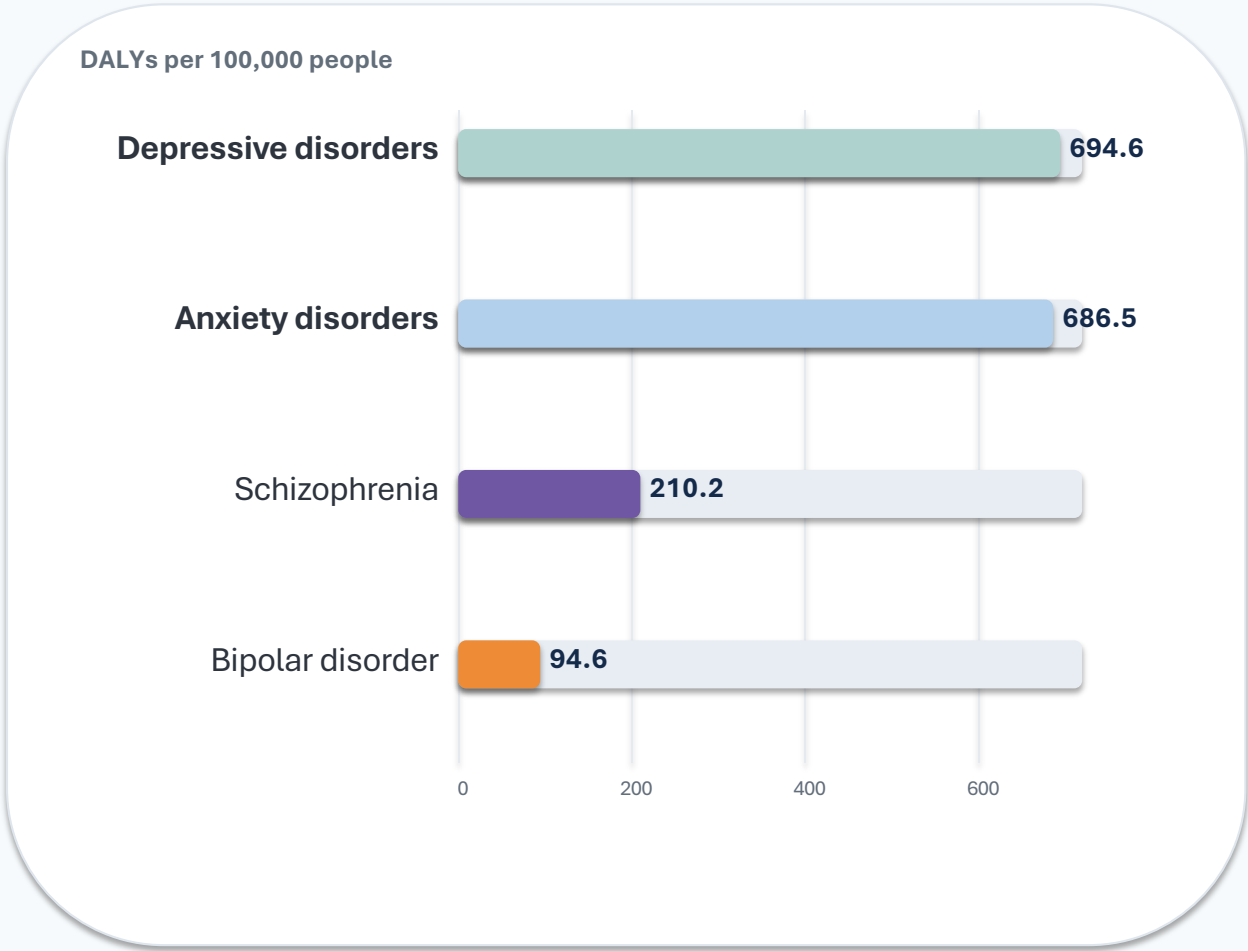
Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide

The two largest categories differ by only 8.1 DALYs per 100,000 people.



Lower Prevalence, Severe Individual Impact

Schizophrenia and bipolar disorder have lower population-level DALYs than depression or anxiety, but severe disability.



Schizophrenia**210.2**

Less common, yet associated with severe disability and a substantial population burden.

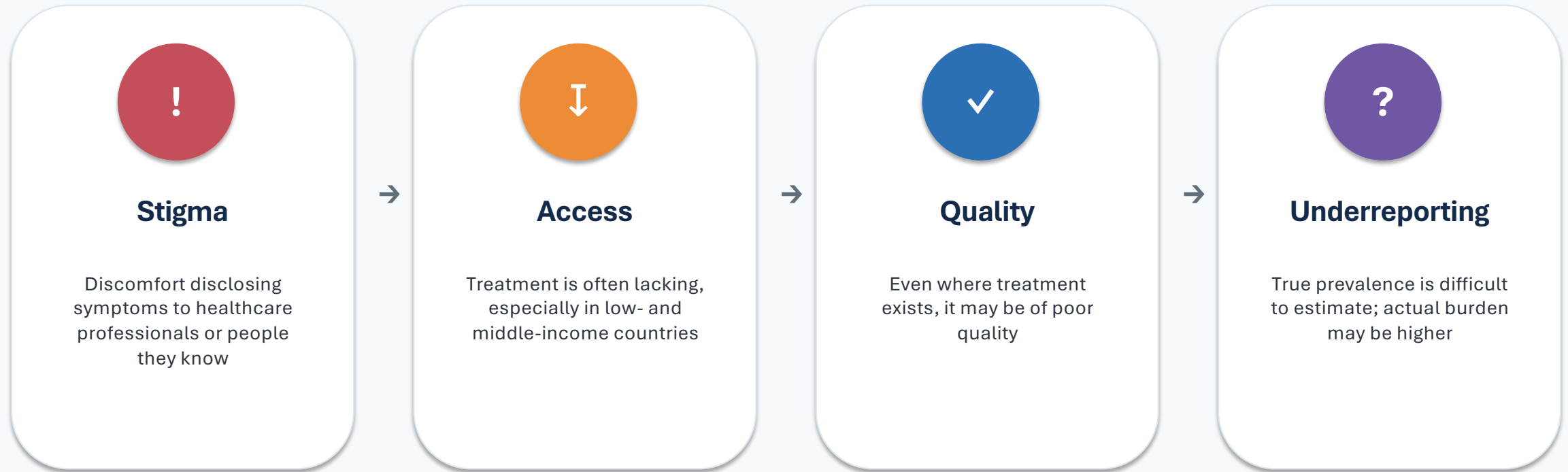
Bipolar disorder**94.6**

Smaller but meaningful share of measured mental health disease burden.

Implication: scale common-disorder care without displacing specialized pathways for severe conditions.

Treatment Is Available but Often Lacking or Poor Quality

Mental illnesses are treatable, yet barriers limit adequate care and weaken the data picture.



Good data is essential to understand how, when, and why mental health conditions occur — and how they can be treated effectively and safely.

How People Cope: Medication, Faith, and Family Conversations

Global survey data shows responses are shaped by cultural, economic, and healthcare-system factors.



Prescribed medication

Common in higher-income settings



Religious or spiritual activities

Prevalent in many regions worldwide



Talking to friends or family

Widely reported coping mechanism

Policy design should account for these differences rather than applying a one-size-fits-all approach.

Figure reference: coping strategies across countries

Key Takeaways for Policy and Program Planning

Prioritize scale for common disorders while maintaining specialized care for severe conditions.

1

Scale common-disorder interventions

Depression and anxiety together represent the overwhelming majority of mental health DALYs across the five measured categories.

2

Protect severe-condition care pathways

Schizophrenia and bipolar disorder carry disproportionate individual burden and require specialized support.

3

Close the treatment gap

Treatment exists, but access and quality remain inadequate in many settings.

4

Reduce stigma and improve measurement

Stigma suppresses both help-seeking and accurate estimates of prevalence.

5

Design for local coping patterns

Coping strategies vary widely across countries and cultures, requiring context-sensitive responses.

Program priority: expand evidence-based services for depression and anxiety while sustaining high-quality care for severe mental illness.